



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : رشا نيازى فرحات الحمصى

العنوان : بهناى

الحي / المدينة : الباجور

المحافظة : المنوفيه

تاريخ تسجيل الإستمارة : 04-10-2026

رقم البطاقة : 27810011703449 رقم التليفون : 01022536597

العمر (تاريخ الميلاد) : 01-10-1978 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي : _____
التوقيع: (_____)

المقرر بما فيه : _____
التوقيع: (_____)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : _____

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان

الوظيفة : _____
الوظيفة : _____
الوظيفة : _____

التوقيع : _____
التوقيع : _____
التوقيع : _____

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي : _____
خطة العلاج : _____
المستشفى : _____
جهة العلاج : _____
ملاحظات : _____

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: رشا نيازي فرحات الحمصي
National ID: 27810011703449



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :

PM 01:17 05-07-2026

تاريخ وساعة الدخول :

---:--- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المريضة تعاني من ورم غير حميد بالبدن و تحتاج الي Further investigations and imaging

الوقت :

---:---

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاثنين





Name: رشا نبارى فرحات الحمصى			
MR. : 12350			
Detailed Medical Report			
			السيد
تحية طيبة وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننتشرف بأن نوضح فيما يلي تقريراً عن حالته			
		تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 47-year-old post-menopausal individual with a negative family history and no reported co-morbidities. The clinical history notes a left-sided invasive ductal carcinoma (IDC) classified as triple-negative breast cancer (TNBC) previously managed with a simple hysterectomy and unilateral salpingo-oophorectomy in 2017 for fibroids, followed by a completion salpingo-oophorectomy in 2023. Pathology from the left breast core biopsy confirms left IDC grade 3. Hormone-receptor testing is negative (ER 0/8, PR 0/8) and HER2 is negative (0). The proliferation index is KI67 40 %. Imaging with sonomammography (ACR b) demonstrates a homogeneous fibroglandular parenchyma and an irregular, hypo-echoic, non-parallel mass in the left breast at the 2 o'clock position measuring 15 × 12 mm. No macro- or micro-calcifications are identified. The skin, nipple-areola complex, lactiferous ducts, and subcutaneous tissue are unremarkable. Axillary assessment shows a few enlarged left axillary lymph nodes, the largest 13 × 10 mm with a globular pattern, loss of fatty hilum, while no pathological nodes are seen in either axilla. Metastatic workup reveals retro-pectoral lymphadenopathies on CT chest-abdomen-pelvis, with a clear chest X-ray, bone scan, and PAUS, and a clear operative bed. The referring team recommends consideration of tumor clipping, neoadjuvant chemotherapy (NACT) followed by left breast-conserving surgery (BCS) with sentinel lymph node biopsy (SLNB) according to response, with accompanying photographic documentation and BSM film provided to the MDT. The MDT plan includes contrast-enhanced spectral mammography (CESM) and clipping, echocardiography, and placement of a porta-cath. Neoadjuvant therapy will commence with Keytruda (pembrolizumab) as per KEYNOTE-522 combined with taxol and carboplatin for 12 weeks, followed by four cycles of AC (doxorubicin-cyclophosphamide) with Keytruda. Subsequent management will be left BCS with SLNB contingent on treatment response.			
تحريراً في ,			
		التاريخ :	23-09-2026
		الوقت :	
		الطبيب المعالج :	
يعتمد ,			



Chp L
A-14



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



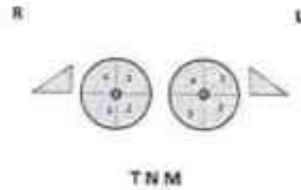
08/13/2026

MDT FORM

Name: رشا تباري لرحمات الحمصى	MRN: 12350
Age: 48 Y	ID: 27810011703449
Menopausal Sta: POST	Phone: 01011305410
NRS SCORE:	Marital Status: MARRIED
BMI:	Family History: -VE
Gender:	Reason Of MDT: NEW
CO Morbidity: FREE	ECOG Performance Status 0 1 2 3 4
Surgical History: * OPEN SIMPLE HYSTERECTOMY+UNI LATERAL SO 2017, THEN COMPLETION SO 2023 (BENGIN)	

Clinical Data:

BX: LT IDC G3
ER: 0/8 PR: 0/8 HER2: 0 KI67: 40%



History:

48YO LT IDC TNBC, UNDERWENT SIMPLE HYSTERECTOMY + UNILATERAL SO 2017 FOR FIBROIDS THEN COMPLETION SO 2023

Sonomammography:

ACR (b).

- * Normal skin and nipple.
- Complementary ultrasound revealed:
- * Homogenous fibroglandular breast parenchyma is noted.
- * Left breast an irregular, hypochoic non-parallel mass lesion is noted at 2 o'clock measuring 15x12 mm.
- * Normal lactiferous ducts.
- * No definite breast macro or microcalcifications could be noted.
- * Normal skin and subcutaneous tissue.
- * Normal breast nipple and areola complex.
- * Normal axilla with no pathological lymph nodes could be seen.
- * Normal right axilla with no pathological lymph nodes could be seen.
- * Few enlarged left axillary lymph nodes; the largest measures (13x10 mm), show globular pattern with hypochoic cortex and lost fatty hilum.

Biomarker/Molecular:

TNBC

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

LT T2 N1 M0

Pathological Staging:

Mets Workup:

CT CAP :RETROPECTORAL LYMPHATENOPATHIES

CXR:FREE

BONE SCAN FREE

PAUS:FREE PARAMETRIA ,CLEAR OPERATIVE BED

Recommendations:

FOR POSSIBILITY OF CLIPPING NACT THEN LT BCS +SLNB ACC TO RESPONSE

N:B PHOTOS ATTACHED

BSM FILM HANDED TO MDT

MDT Outcome & Plan

CESM / CLIPPING

ECHO / PORTA-CATH

START NACT (KEYNOTE 522) TAXOL / CARB 12WKS - AC X 4C WITH keytruda

then LT BCS +SLNB ACC TO RESPONSE

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

Patient Name	رشا تيزي فرحات	Case No.	342240
Sex/Age	Female 47 Y	Patient No.	24810
Registered Date	15/7/2026 01:52:23 PM	Referred By Dr.	Out Patient
Validation Date	11/8/2026 12:22:06 PM	Branch	Gustave Roussy
Reporting Date	11/8/2026 12:24:50 PM	Pathology No.	SR-811/26

Breast Slide Revision

Clinical Data: Left breast mass.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

- One referred block labelled by (27545). Together with, five referred slides including 1 (H&E) stained slide and 5 immunostained slides for (ER, PR, Her2 & Ki67).

Microscopic Examination:

- * Histological type: Invasive micropapillary carcinoma.
- * **Grade:** 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * Microcalcification: Absent.

IPT:

- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Negative incomplete membrane reaction in tumor cells (score 0).
- Ki67%: 40%.

Diagnosis:

Left breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 3.

IPT:

- ER: Negative (0/8).
- PR: Negative (0/8).
- Her2: Negative (score 0).
- Ki67%: 40%
- * **Scoring system:** "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, $\geq 1\%$; 2, between 1%





Gustave Roussy International | Egypt - Pathology Lab
جوستاف روسي انترناتيونال - مصر - مختبر الأمراض



Patient Name	رشا نيازي فرحات	Case No.	342240
Sex/Age	Female 47 Y	Patient No.	24820
Registered Date	15/7/2026 01:52:23 PM	Referred By Dr.	Out Patient
Validation Date	11/8/2026 12:22:06 PM	Branch	Gustave Roussy
Reporting Date	11/8/2026 12:24:50 PM	Pathology No.	SR-811/26

and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

***Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Dr. Walaa Ibrahim Ghoneim, M.D



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mide El Qadima - Cairo, Egypt

Name : RASHA Niazi ALHMSI	Pat. ID: 302841
Date : 02/07/2026	Ref. Doctor: عاصم فايد

Pelviabdominal US

-Liver:

Average in size measures 15 CM in its cranio-caudal diameter(N up to 15 cm in MCL),It displays homogenous echopattern with smooth outline.no periportal fibrosis.no focal lesions or dilated Intra or extra-hepatic biliary radicals could be detected.the hepatic veins are of normal calibre with regular course

-PV: is echofree patent lumen with no evidence of thrombus formation,of average calibre measuring 9mm in diameter(N up to 13mm) with normal monophasic hepatopedal flow & MAX. V measuring 18cm/sec (Normal range 12-25 cm/sec.

-Biliary system:

- **G.B:** Average wall thickness measuring 2 mm (N up to 3MM) showing echo-free cavity with no stones,mud or pathological growth could be detected.
- **CBD** is of average caliber measuring 4mm in diameter

-Spleen:

11 cm in its longitudinal axis (N up to 13cm. In MAL),It displays homogenous echopattern with no focal lesion.the splenic vein is not dilated measuring 5mm

-Both kidneys:

Are of average size,showing average cortical thickness with good cortico-medullary differentiation.No stones or backpressure could be detected.No masses.Free peri-nephric space.no suprarenal masses

RK measuring 10.5 x4.5 cm with cortical thickness measuring 18mm

LK measuring 11x5 cm with cortical thickness measuring 20mm

-Pancreas: no lesions, calcifications, cysts or dilated pancreatic duct could be detected.

-para-aortic area:are concealed by gases.

-UB:Normal wall thickness. Normal shape & contour.. No diverticulae,masses or stones.

-hysterectomy done ,clear parametrium

-No free fluid

-average small bowel motions and thickness,average colon

conclusion

- **Normal study .**

DR,hatem moharram

اسم المريض بالإنجليزية: rasla neyaze	كود المريض: 27810011703449
تاريخ التقرير: 2025/08/17 11:51:54 AM	الجنس: ♀ سنة: 40 شهر: 1

Ultrasound Guided clip insertion:

Clinical summary: Pathologically proven Left breast invasive ductal carcinoma grade III for clip insertion.

Technique:

- Under complete sterile conditions, 3ml of local anaesthetic were administered at the site of concern of the Left breast.
- one metallic clip was inserted within the center of large malignant mass lesion and measures about 15x12mm and located at 2 o'clock position.
- This was confirmed by mammography CC & MLo views.

Best Regards
Dr. sara gamal, MSc

Dr. M. Nabil El Bolkainy

Ph. D. (Path.) Michigan, U.S.A.

Dr. Tarek N. El Bolkainy

Ph. D. (Path.) Cairo University



دكتور محمد نبيل البلقيني

دكتوراد الباثولوجيا - جامعة ميشيجان

دكتور طارق نبيل البلقيني

دكتوراد الباثولوجيا - جامعة القاهرة

Name: رشا نيازي فرحات Age : 40 Years Sex: Female

Clinical: Total hysterectomy & unilateral salpingo-oophorectomy.

Referred By: Prof

Date: 17-اكتوبر-17

Gross

Total hysterectomy with unilateral salpingo-oophorectomy specimen showing corpus uteri 7x6x6 cm with endometrial thickness 3 cm, cervix is 3 cm long, ovary is 3x3 cm.

Microscopic

- * The endometrium shows increased gland density relative to the stroma (gland to stroma ratio >1). The glands are dilated tubular type lined by stratified epithelium with few mitoses. The glands also show disorganized architectural distribution, but separated by distinct hyperplastic stroma. No evidence of complex pattern.
- * There are associated islands of hyperplastic endometrial glands and stroma among the smooth muscle bundles of superficial myometrium.
- * The endocervix shows cystic glands filled with mucin. There is associated mild nonspecific inflammation in the stroma. No evidence of cellular atypia or malignancy. Ovary is unremarkable. (MH)

Diagnosis

Uterus, Total hysterectomy & unilateral salpingo-oophorectomy: Simple endometrial hyperplasia + Adenomyosis + Nabothian cysts.

Category

Hyperplasia.

{code: 10237 / 51 }

Tarek N. El Bolkainy

M. N. El Bolkainy, M.D. Ph. D.
Date: 19/10/2017

Pathology Report



J6077-23

Patient Name: رشا نيازي الحمصي Age: 46 years Gender: Female
Referred by: *Prof. Dr. Mohammed Mostafa*
Date of Receipt: 8/6/2023 Result released on: 12/6/2023

Present History: Left ovarian mass.
Specimen: Left oophrectomy.

Gross Morphology:

Received opened ovarian cystic mass measured 4x3x3 cm.

Microscopic Morphology:

Sections examined from ovarian cyst lined by festooning luteinized granulosa and theca cells. Adjacent ovarian tissue showed unremarkable pathologic changes.

There was no stratification, atypia, mitosis, necrosis, stromal invasion or malignancy.

Impression:

Laparoscopic ovarian cystectomy:

Non Neoplastic Cyst, Corpus Luteum cyst

- Kindly correlate with clinical and radiological data.

Consultant Pathologist
Dr. Rehab Samaka
Prof. Dr. Rehab Monir Samaka, MD





Patient Name: م. ج. ج.	Patient ID: 495181
Study Date: 04-Jul-26 11:00:00 AM	

CT CHEST, ABDOMEN & PELVIS WITH CONTRAST:

I) Chest:

- Left breast UOQ soft tissue nodule is seen measuring about 1.5 cm.
- Sub-jacent deep two nodules are seen measuring up to 1 cm.
- Multiple enlarged left axillary level I, II & III LNs are seen measuring up to 2.5 cm.
- Clear both lungs showing no masses, nodules, cavitary lesion, consolidations, or bronchiectatic changes.
- Small lingular air filled cyst is seen.
- No pleural or pericardial effusion.
- No gross cardiac abnormality.
- No mediastinal or hilar lymphadenopathy.
- Small hiatus hernia.
- Residual thymic tissue noted.

II) Abdomen & pelvis:

- Average sized liver showing smooth outlines with no focal lesions or dilated IHBRS. Patent main portal vein and its central branches.
- Normal CT appearance of the spleen (a splenule is seen), adrenal glands, pancreas, kidneys, IVC and aorta.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Evidence of sub-total hysterectomy.
- Normal other pelvic organs.

III) Skeleton:

- No lytic or sclerotic osseous lesions seen within the scanned skeleton.

Conclusion:

- *Reported case of breast carcinoma showing left breast ST tissue nodules; with multiple axillary & retro-pectoral lymphadenopathies.*

Dr. Mohamed Abdelateef, MD

Nuclear Medicine Unit

Patient Name: Rasha Niazy Farahat

Date: 5/7/2026

Referring physician: Prof.

^{99m}Tc-MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP.

Report:

- Heterogenous radiotracer uptake is seen at the spine, more advanced at the lower lumbar vertebrae likely of marked spondylo-dgenerative changes .
- Otherwise, no definite hot spot that accounts for osseous deposits could be detected all over the surveyed skeleton..
- Osteoarthritic changes are seen at both shoulders, both knees .
- Both kidneys are normally seen.

Conclusion:

The surveyed skeleton is free of any active osseous lesions

Thanks for referral

Nuclear Medicine Unit

DR. Sherin Mady, MD

DR Mohamed Elmaghrabi

ECHO REPORT

NAME: علي بن علي

DATE: 6 / 9 / 2026

ID NUMBER: /

SEX: AGE:

EF	65%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.3	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVEDD	2.1	2-4cm	LA	3.6	2-4cm
FS	30%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function Normal

SWMA Normal RMA

LA and Ao Root Dimensions Normal

Mitral Valve Normal

Aortic Valve Normal

Tricuspid Valve Normal

Pulmonary Valve Normal

Right Side and Systolic Function Normal

Intracardiac Masses and Effusion Normal

DR:

علي بن علي

ECHO REPORT

NAME: علي بن علي

DATE: 6 / 9 / 2026

ID NUMBER: /

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DR:

Ali

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DATE: 6 / 9 / 2026

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LV Diastolic Function Normal

SWMA Normal RWA

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Tricuspid Valve Normal

Pulmonary Valve Normal

Right Side and Systolic Function Normal

Intracardiac Masses and Effusion Normal

DR: Ali

اسم المريض: <i>فاطمة هريزه</i>	7781001703419	كود المريض:
تاريخ الفحص: 2026/08/17 11:51:54 AM	قسم الفحص:	قسم الفحص:

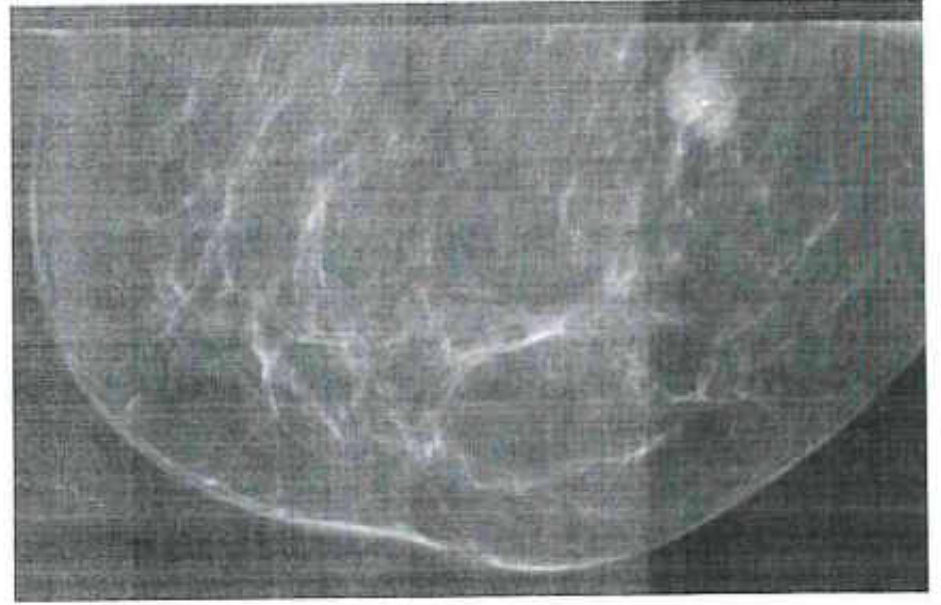
Ultrasound Guided clip insertion:

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دكتور محمد نبيل البلقيني
دكتوراد الباثولوجيا - جامعة ميشيغان
دكتور طارق نبيل البلقيني
دكتوراد الباثولوجيا - جامعة القاهرة

Name: رشا نبيل فرحات

Age : 40 Years Sex: Female

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- * The endometrium shows increased gland density relative to the stroma (gland to stroma ratio >1). The glands are dilated tubular type lined by stratified epithelium with few mitoses. The glands also show disorganized architectural distribution, but separated by distinct hyperplastic stroma. No evidence of complex pattern.
- * There are associated islands of hyperplastic endometrial glands and stroma among the smooth muscle bundles of superficial myometrium.
- * The endocervix shows cystic glands filled with mucin. There is associated mild nonspecific inflammation in the stroma. No evidence of cellular atypia or malignancy. Ovary is unremarkable. (MH)

Diagnosis

Uterus, Total hysterectomy & unilateral salpingo-oophorectomy: Simple endometrial hyperplasia + Adenomyosis + Nabothian cysts.

Category

Hyperplasia.

Tarek N. El Bolkainy

M. Nabil El Bolkainy

{code: 10237 / 51 }

M. Nabil El Bolkainy, M.D. Ph. D.
Date: 19/10/2017

Pathology Report



J6077-23

Patient Name: رشا نيازى الحمصى

Age: 46 years

Gender: Female

Referred by: *Prof. Dr. Mohammed Mostafa*

Date of Receipt: 8/6/2023

Result released on: 12/6/2023

Present History: Left ovarian mass.

Specimen: Left oophrectomy.

Gross Morphology:

Received opened ovarian cystic mass measured 4x3x3 cm.

Microscopic Morphology:

Sections examined from ovarian cyst lined by festooning luteinized granulosa and theca cells. Adjacent ovarian tissue showed unremarkable pathologic changes.

There was no stratification, atypia, mitosis, necrosis, stromal invasion or malignancy.

Impression:

Laparoscopic ovarian cystectomy:

Non Neoplastic Cyst, Corpus Luteum cyst

- Kindly correlate with clinical and radiological data.

Consultant Pathologist
Dr. Rehab Samaka
Prof. Dr. Rehab Monir Samaka, MD





د. هـ - ٨



مبادرة رئيس الجمهورية
لحصد صحة المرأة المصرية

الهيئة العامة
للصحة العامة

08/13/2026

MDT FORM

Name:	د. هـ - ٨	MRN:	12350
Age:	48 Y	ID:	27810011703449
Menopausal Sta:	POST	Phone:*	01011305410
NRS SCORE:		Marital Status:	MARRIED
BMI:		Family History:	-VE
Gender		Reason of MDT:	NEW
CO Morbidity:	FREE	ECOG Performance Status	0 @ 10 20 30 4
Surgical History: * OPEN SIMPLE HYSTERECTOMY+UNI LATERAL SO 2017, THEN COMPLETION SO 2023 (BENGIN)			

Clinical Data:

BX:

LT IDC G3

ER:

0/8

HER2: 0

KI67: 40%

R

L



T N M

History:

48YO LT IDC TNBC, UNDERWENT SIMPLE HYSTERECTOMY +UNI LATERAL SO 2017 FOR FIBROIDS THEN COMPLETION SO 2023

Genitotamography:

ACR (b).

- * Normal skin and nipple.
- * Complementary ultrasound revealed:
- * Homogenous fibroglandular breast parenchyma is noted.
- * Left breast an irregular, hypochoic non-parallel mass lesion is noted at 2 o'clock measuring 15x12 mm.
- * Normal lactiferous ducts.
- * No definite breast macro or microcalcifications could be noted.
- * Normal skin and subcutaneous tissue.
- * Normal breast nipple and areola complex.
- * Normal axilla with no pathological lymph nodes could be seen.
- * Normal right axilla with no pathological lymph nodes could be seen.
- * Few enlarged left axillary lymph nodes; the largest measures (13x10 mm), show globular pattern with hypochoic cortex and lost fatty hilum.

Biopsy/Molecular

TNBC

Clinical Staging:

LT T2 N1 M0

Pathological Staging:

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Meta Working:

CT CAP :RETROPECTORAL LYMPHADENOPATHIES

CXR:FREE

BONE SCAN FREE

PAUS:FREE PARAMETRIA ,CLEAR OPERATIVE BED

Recommendations:

FOR POSSIBILITY OF CLIPPING NACT THEN LT BCS +SLNB ACC TO RESPONSE

N:8 PHOTOS ATTACHED

BSM FILM HANDED TO MDT

MDT Outcome & Plan

CESH / CLIPPING

ECHO / PORTA-CATH

START NACT (KEYNOTE 522) TAXOL / CARB 12WXS - AC X 4C WITH keytruda

then LT BCS +SLNB ACC TO RESPONSE

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

Patient Name	رشا نيزي فرحات	Case No.	342240
Sex/Age	Female 47 Y	Patient No.	24810
Registered Date	15/7/2026 01:52:23 PM	Referred By Dr.	Out Patient
Validation Date	11/8/2026 12:22:06 PM	Branch	Gustave Roussy
Reporting Date	11/8/2026 12:24:50 PM	Pathology No.	SR-81/26

Breast Slide Revision

Clinical Data: Left breast mass.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

- One referred block labelled by (27545). Together with, five referred slides including 1 (H&E) stained slide and 5 immunostained slides for (ER, PR, Her2 & Ki67).

Microscopic Examination:

- * Histological type: Invasive micropapillary carcinoma.
- * Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Angiolymphatic invasion: Absent.
- * Perineural invasion: Absent.
- * Microcalcification: Absent.

IPT:

- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Negative incomplete membrane reaction in tumor cells (score 0).
- Ki67%: 40%.

Diagnosis:

Left breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 3.

IPT:

- ER: Negative (0/8).
- PR: Negative (0/8).
- Her2: Negative (score 0).
- Ki67%: 40%
- * Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%, 2, between 1%



Patient Name	رشا نبازي فرحات	Case No.	342240
Sex/Age	Female 47 Y	Patient No.	24820
Registered Date	15/7/2026 01:52:23 PM	Referred By Dr.	Out Patient
Validation Date	11/8/2026 12:22:06 PM	Branch	Gustave Roussy
Reporting Date	11/8/2026 12:24:50 PM	Pathology No.	SR-811/26

and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

****Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Dr. Walaa Ibrahim Ghoneim, M.D



خريطة رقم (64) - (البريد الإلكتروني) - مبنى (الطابق الثاني) - مبنى (الطابق الثاني) - مبنى (الطابق الثاني) - مبنى (الطابق الثاني)
Room No. (4) - First Floor-Building No. 9677, Site Corniche - Alvor Et Qadima - Cairo, Egypt

Name : RASHA Niazi ALHMSI	Pat. ID: 302841
Date : 02/07/2026	عاصم فاني Ref. Doctor:

CXR:

No nodules or cavities
Average heart size
Normal c/p angles
Normal paratracheal stripe
Average bony cage
Dr hatem moharram

Name : RASHA Niazi ALHMSI	Pat. ID: 302841
Date : 02/07/2026	Ref. Doctor: عاصم فايد

Pelviabdominal US

-Liver:

Average in size measures 15 CM in its cranio-caudal diameter(N up to 15 cm in MCL),It displays homogenous echopattern with smooth outline.no periportal fibrosis.no focal lesions or dilated Intra or extra-hepatic biliary radicals could be detected.the hepatic veins are of normal calibre with regular course

-PV: is echofree patent lumen with no evidence of thrombus formation,of average calibre measuring 9mm in diameter(N up to 13mm) with normal monophasic hepatopedal flow & MAX. V measuring 18cm/sec (Normal range 12-25 cm/sec.

-Biliary system:

- **G.B:** Average wall thickness measuring 2 mm (N up to 3MM) showing echo-free cavity with no stones,mud or pathological growth could be detected.
- **CBD** is of average caliber measuring 4mm in diameter

-Spleen:

11 cm in its longitudinal axis (N up to 13cm. In MAL),It displays homogenous echopattern with no focal lesion.the splenic vein is not dilated measuring 5mm

-Both kidneys:

Are of average size,showing average cortical thickness with good cortico-medullary differentiation.No stones or backpressure could be detected.No masses.Free peri-nephric space.no suprarenal masses

RK measuring 10.5 x4.5 cm with cortical thickness measuring 18mm

LK measuring 11x5 cm with cortical thickness measuring 20mm

-Pancreas: no lesions, calcifications, cysts or dilated pancreatic duct could be detected.

-para-aortic area:are concealed by gases.

-UB:Normal wall thickness. Normal shape & contour.. No diverticulae,masses or stones.

-hysterectomy done ,clear parametrium

-No free fluid

-average small bowel motions and thickness,average colon

conclusion

- **Normal study .**

DR,hatem moharram

^{99m}Tc-MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- Heterogenous radiotracer uptake is seen at the spine , more advanced at the lower lumbar vertebrae likely of marked spondylo-dgenerative changes .
- Otherwise , no definite hot spot that accounts for osseous deposits could be detected all over the surveyed skeleton..
- Osteoarthritic changes are seen at both shoulders , both knees .
- Both kidneys are normally seen.

Conclusion:

The surveyed skeleton is free of any active osseous lesions

Thanks for referral

Nuclear Medicine Unit

DR . Sherine Mahdy, MD

DR/ Mohamed Emam

د. محمد إمام
٢٠٢٥ / ٧ / ٥



Patient Name: رشا نيازى فرحات	Patient ID: 495181
Study Date: 04-Jul-26 11:00:00 AM	

CT CHEST, ABDOMEN & PELVIS WITH CONTRAST:

I) Chest:

- Left breast UOQ soft tissue nodule is seen measuring about 1.5 cm. Sub-jacent deep two nodules are seen measuring up to 1 cm.
- Multiple enlarged left axillary level I, II & III LNs are seen measuring up to 2.5 cm.
- Clear both lungs showing no masses, nodules, cavitory lesion, consolidations, or bronchiectatic changes.
- Small lingular air filled cyst is seen.
- No pleural or pericardial effusion.
- No gross cardiac abnormality.
- No mediastinal or hilar lymphadenopathy.
- Small hiatus hernia.
- Residual thymic tissue noted.

II) Abdomen & pelvis:

- Average sized liver showing smooth outlines with no focal lesions or dilated IHBRs. Patent main portal vein and its central branches.
- Normal CT appearance of the spleen (a splenule is seen), adrenal glands, pancreas, kidneys, IVC and aorta.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Evidence of sub-total hysterectomy.
- Normal other pelvic organs.

III) Skeleton:

- No lytic or sclerotic osseous lesions seen within the scanned skeleton.

Conclusion:

- **Reported case of breast carcinoma showing left breast ST tissue nodules; with multiple axillary & retro-pectoral lymphadenopathies.**

د/ شريف نبيل فافل
نائب إدارى قسم الأشعة

Dr. Mohamed Abdelateef. MD

Mammography and complementary Ultrasonography report of the both breast revealed:-

Mammography revealed:

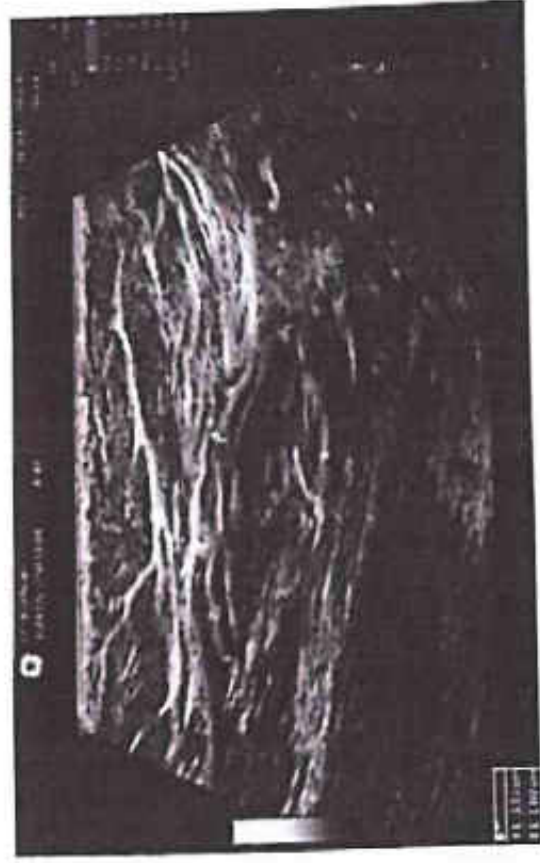
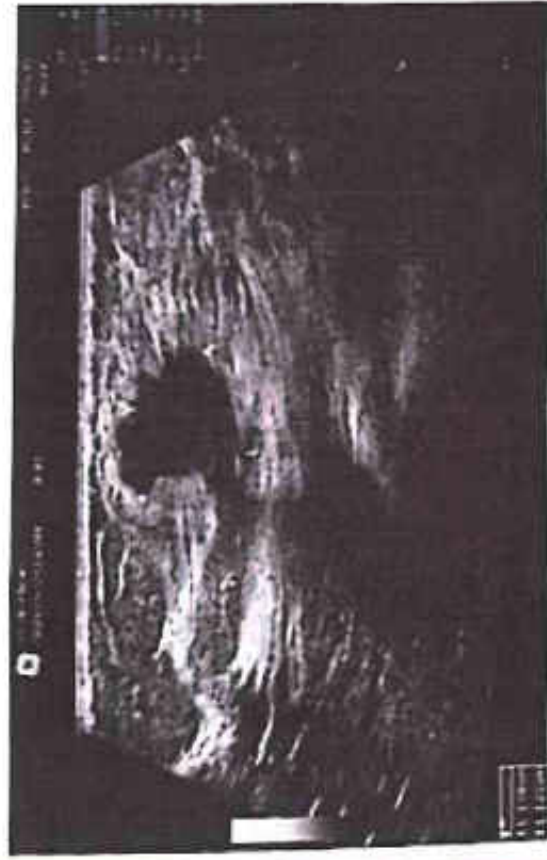
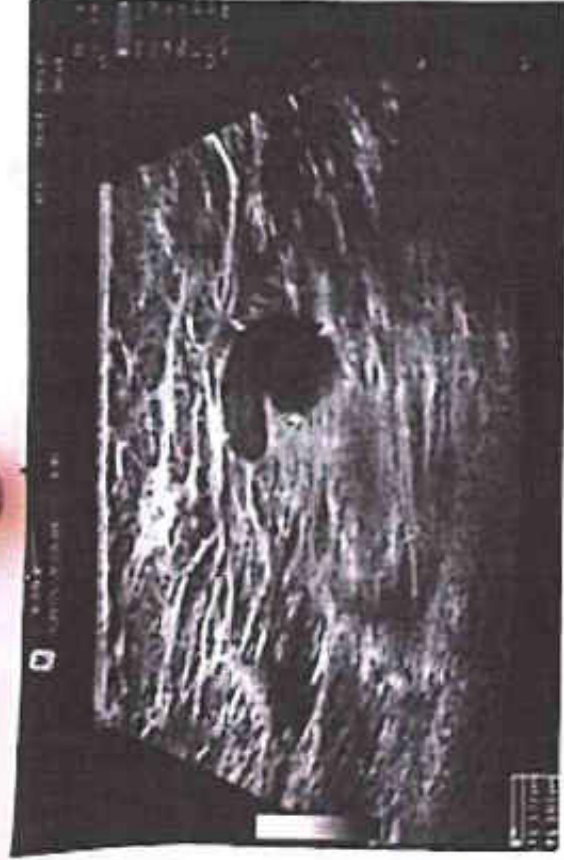
- Breast composition is scattered fibroglandularACR (b).
- No definite breast macro or microcalcifications could be noted.
- Normal skin and nipple.
- Complementary ultrasound revealed:
- Homogenous fibroglandular breast parenchyma is noted.
- Left breast an irregular, hypoechoic non-parallel mass lesion is noted at 2 o'clock measuring 15x12 mm .
- Normal lactiferous ducts.
- No definite breast macro or microcalcifications could be noted.
- Normal skin and subcutaneous tissue.
- Normal breast nipple and areola-complex.
- Normal axilla with no pathological lymph nodes could be seen.
- Normal right axilla with no pathological lymph nodes could be seen.
- Few enlarged left axillary lymph nodes; the largest measures (13x10 mm), show globular pattern with hypoechoic cortex and lost fatty hilum.

Conclusion

- Left breast suspicious mass as described....BIRADS 4 classification
- Left axillary suspicious LNs as described.
- Negative right breast ... BIRADS 1 classification.

Examined and reported by
Dr. Estera Rasheed, MSc
Dr. Estera Yousef.





Code: 27545

Sex: Female

التاريخ: 21- 6 - 2026 م

اسم المريض: رشا نيازي الحمصي

السن: 49 سنة

الطبيب المعالج: المستشفى الجامعي

Clinical Data: Left breast mass.

Pathology report

Nature of the Specimen:

Paraffin block.

Gross Appearance:

Sections immunostained for c-erb-2 (HER2/neu) using supersensitive monoclonal antibodies with Dako Envision (dextran substrate polymer) detection system. Prior microwave treatment for antigen retrieval and blocking of internal peroxidase was performed. DAB was utilized as a chromogen.

Microscopic Appearance:

For the determination of HER2 protein overexpression, only the membrane staining intensity and pattern in the invasive part of tumor are evaluated. HER2 overexpression is interpreted as negative for both scores 0 (no staining is observed *or* membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells) and 1+ (faint/barely perceptible incomplete membrane staining in $> 10\%$ of tumor cells). The test is interpreted as weakly positive (border line or equivocal) for score 2+ (circumferential membrane staining that is incomplete and/or weak/moderate within $> 10\%$ of the invasive tumor cells *or* complete and circumferential membrane staining that is intense and within $\leq 10\%$ of the invasive tumor cells). A strong positive result is interpreted when the score is 3+ (strong circumferential membrane staining in $> 10\%$ of invasive tumor cells).

Diagnosis:

IMMUNOSTAINING FOR C-ERB-2 (HER2/NEU):

- HER2/neu status: Negative (score 0)

د. أسامة مصطفى
مستشفى الجامعة
الأورام

01021006137 - 01096132618

شبين الكوم - ميدان شروق - شارع اسكندر متفرع من شارع الأيمن - بوج الانوار الجديدة - الدور الثالث

Code: 27545

Sex: Female

التاريخ: 21- 6- 2026م

اسم المريض: رشا تيارى الحمصى

السن: 49 سنة

الطبيب المعالج: المستشفى الجامعي

Clinical Data: Left breast mass.

Pathology report

Nature of the Specimen:

Paraffin block.

Gross Appearance:

Sections immunostained for ER, PR and Ki-67 using supersensitive monoclonal antibodies with Dako Envision (dextran substrate polymer) detection system. Prior microwave treatment for antigen retrieval and blocking of internal peroxidase was performed. DAB was utilized as a chromogen.

Microscopic Appearance:

ER: Tumor cells show no nuclear staining (*Allred scoring system: 0/8*).

PR: Tumor cells show no nuclear staining (*Allred scoring system: 0/8*).

Ki-67: 50 % of tumor cells (*at the hot spots*) show strong nuclear staining.

Diagnosis:

ER, PR IMMUNOHISTOCHEMISTRY:

- **ER Status:** *Negative* (*Allred scoring system: 0/8*)

- **PR Status:** *Negative* (*Allred scoring system: 0/8*)

KI-67 IMMUNOHISTOCHEMISTRY:

- **Ki-67 labeling index:** 50 %.

علاء الدين مصطفى
مستشار
الأورام والأورام

© 01021006137 - 01096132618

شيبان الكسوم - ميدان شرف - شارع السكندر متفرع من شارع الأمين - برج الأنوار الجديدة - الدور الثالث

Code: 27545

Sex: Female

التاريخ: 6 - 8 - 2026 م

اسم المريض: رشا دياز ي الحمصي
السن: 49 سنة
الطبيب المعالج: المستشفى الجامعي

Clinical Data: Left breast mass.

Pathology report

Nature of the Specimen:

U/S guided Tru cut biopsy.

Gross Appearance:

Specimen contained multiple grayish tissue cores ranging: 11-15 mm long, totally submitted.

Microscopic Appearance:

Sections reveal a breast tissue infiltrated by moderately differentiated invasive ductal carcinoma composed of nests and cords of cells with ductal differentiation, surrounded by stromal desmoplastic reaction. Tumor cells are large, pleomorphic with moderate anaplasia and mitotic activity. There is associated mild lymphocytic infiltrate. There is no associated significant intraductal component (DCIS).

Diagnosis:

LEFT BREAST MASS; U/S GUIDED TRU CUT BIOPSY:

- *Invasive duct carcinoma, grade II.*

د. هبة محمد مصطفى
الطبيب الأخصائي والأورام

01021006137 - 01096132618

بن الكوم - ميدان شرف - شارع اسكندر متفرع من شارع الأمين - برج الانوار الجديدة - الدور الثالث



Patient Name السيد رشاد نوري فرحات Patient ID. 1026060210042
Age / Sex 47 Years / Female Request Date 02-Jun-2026 07:36 PM
Referred By Himself Printed In 02-Jun-2026 08:00 PM

HEMATOLOGY REPORT

Reference Range:

Blood Picture:					
Δ Hgb (Hemoglobin)	: 11.9	g/dL	L	(13 : 17)	
Δ RBCs (Red Blood Cells)	: 4.7			3.8 - 5.3 x 10 ¹² Cells/ μ L	
Δ Hct (Hematocrit)	: 35	%		35 - 47 %	
Δ MCV	: 74.47	fL/cell	L	80 - 100 fL/cell	
Δ MCH	: 25.32	pg/cell	L	27 - 34 pg/cell	
Δ MCHC	: 34	g/dL		32 - 37 g/dL	
Δ RDW	: 13.2	%		11.5 - 14.5 %	
Δ Plt (Platelet)	: 264	x 10 ³ /mm ³		150 - 440 x 10 ³ /mm ³	
• Pct	: 0.27	%		0.100 - 0.500 %	
• MPV	: 10.23	fL		8.0 - 12.0 fL	
• PDW	: 15.5	%		10.0 - 18.0 %	
Δ WBCs (Leukocytes)	: 7.3	x 10 ³ /mm ³		4.5 - 11.0 x 10 ³ /mm ³	

Test	Relative count %	Absolute count K/ μ L
• Neutrophil	: 55	4.01
• Lymphocytes	: 37	2.7
• Monocytes	: 7	0.51
• Eosinophils	: 1	0.07
• Basophils	: 0	0
		1.8 - 7.7
		0.8 - 4.8
		0.2 - 0.9
		0 - 0.8
		0 - 0.1

Comment: RBCs show mild Microcytic Hypochromic Anemia, WBCs show no significant abnormality, PLTs show adequacy in number and well aggregated

Doctor's signature:

Thanks

Patient History in Our Lab										
WBCs	RBCs	Hgb	Hct	MCV	MCH	MCHC	Neut	Lymph	Plt	
7.5	4.8	13.1	40	83.33	27.29	32.75	58	37	248	
5.3	4.6	11.6	34	73.91	25.22	34.12	56	35	249	

١٠١٩٦٦٤٤٢٠ - ٢٨٢٠٢٢٥ : ٥
تهنای - الباجور - منوفية



٢٠٢٠/٠١ ٢٧٨١٠٠١١٧٠٣٤٤٩

حاصله على دبلوم تجارة

أنثى مسلمة متروجة

محمد حسين لبيب الجيزي

البطاقة سارية حتى ٢٠٢٧/٠١/٢٦



١٢

